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/ 2026 AHFTC Board Certification Question Bank

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/ Practice Exam - 1

Correct
Answer

Show

Practice Exam - 1

Question 17 of 117



A 78-year-old man with a past medical history of coronary artery disease status-post three vessel CABG, COPD, and high-grade metastatic prostate cancer presents to hospital with progressive dyspnea on exertion and orthopnea. His prostate cancer had been treated with androgen deprivation therapy and enzalutamide until six weeks prior to admission, when he was started on pembrolizumab. Echocardiography the day of admission demonstrates a deterioration in left ventricular ejection fraction from 65% to 25% in the span of three months.

Laboratory evaluation reveals high sensitivity troponin I rising to a peak of 44,802 pg/ml.



Figure 1. ECG on presentation





Figure 2. Baseline ECG from 2 months ago

Which of the following should be considered as a next step in this patient's management plan?

- A. Empiric treatment with prednisone 10 mg daily
- ☒ B. Technetium 99 m pyrophosphate scintigraphy
- ☒ C. Endomyocardial biopsy
- D. Empiric treatment with nonsteroidal anti-inflammatory drugs

Commentary

References

Testing Point: The Board-certified AHFTC specialist should know that immune checkpoint inhibitors are important causes of cardiomyopathy in cancer patients.

Answer: C. Endomyocardial biopsy.

The clinical scenario is that of new onset acute decompensated heart failure, temporally associated with the start of immune checkpoint inhibitor therapy. This class of agents—including ipilimumab, nivolumab, pembrolizumab, cemiplimab, avelumab, atezolizumab, and durvalumab—works by disinhibiting the T cell response. This can result in a variety of cardiotoxicities, most commonly myocarditis and pericarditis. Myocarditis related to immune checkpoint inhibitor therapy exists on a spectrum, ranging from minor elevations in troponin to fulminant disease. When myocarditis related to immune checkpoint inhibitors is suspected, endomyocardial biopsy can be helpful in establishing the diagnosis. The decision to continue or discontinue immune checkpoint inhibitors is often high-stakes, thus definitive diagnosis with tissue is often preferred. Treatments of choice include high-dose steroids or other immune modulating agents such as mycophenolate.

Answer D: NSAIDs are not indicated, nor are low-dose steroids such as

Answer A: prednisone 10 mg daily. The electrocardiographic changes are striking, including new left bundle branch block, and diffuse pseudoinfarction pattern, which can be seen in myocarditis. Left heart catheterization would likely be a part of the diagnostic strategy.

Answer B: The presentation is not consistent with transthyretin amyloidosis, for which technetium-99 m pyrophosphate scintigraphy (technetium PYP scan) may be helpful.

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